POT - PI Oversight Template

Part 1 - Oversight plan

SIMULATED · v1.1

Study / protocol GLENOS-01 / GS-2026-01	**Site / code** Riverside Orthopedic Center / 014	**POT plan ID** POT-014	**Plan revision** v0.2
PI (name) Dr Alex Morgan	**Effective date** 30 Sep 2026	**Linked CCDM record ID / revision** CCDM-014 / v0.2 CCDM-014 / v0.2	**Linked RACI record ID / revision** RACI-014 / r2
Plan prepared by / role Maria Silva / Study Coordinator		**Controlled location of the plan and review records** Investigator site file, tab 2 (Oversight); scans in the site drive /GLENOS-01/POT	

A Purpose and use

The POT is the PI's plan for overseeing the investigation at the site and the record of every oversight review held under that plan. It settles three things before the first participant: which decisions the PI takes personally, which events must reach the PI at once, and how often the PI reviews the site's quality records and with whom. Each review then leaves a written decision on every record read, so the site can show later how the PI exercised oversight.

The POT records decisions and where each action was sent. It does not repeat the content of the records it reads, and it does not follow actions to their end: each action is followed up in the record that receives it.

	Part 1 · Oversight plan	Part 2 · Review record
Sections	A to E	I to III
Completed	Once, before the first participant. Revised only for a reason in Section E.	At every scheduled review and at every review opened by a Section C event. A new copy each time.
By	Plan owner or study coordinator; approved by the PI.	The PI, with the attendees named in Section I.
Reads	CCDM and RACI.	SART, SCML, CSST, SLCT, PIR, FUCR, IMC, CAPA, SRR and escalations received.

Each action goes to the record that owns it: corrective action to the CAPA, and gaps that affect a milestone to the next MRL review.

B1 Judgments reserved for the PI

Mark when the PI acts on each point (**Before** the step, **At decision**, or **After**, by reviewing the record) and the record that shows it. Add the site's own points in the empty lines.

Decision or point	CCDM task ID (if any)	When the PI acts	Evidence of PI involvement (record)
Surgical appropriateness and borderline eligibility	T01	☑ Before ☐ At decision ☐ After	PSEAR decision signed by the PI before enrollment
Interpretation of intraoperative deviations and component substitutions	T03, T04	☐ Before ☑ At decision ☐ After	PI called before wound closure; ICDL deviation entry countersigned
Relation of a complication to the device or the procedure	T02	☐ Before ☑ At decision ☐ After	SART causality section signed by the PI
Early signs of executional drift across cases	T03	☐ Before ☐ At decision ☑ After	SLCT review at the fifth case and at each scheduled review
Interim cover of a key function (RACI Section I)	—	☑ Before ☐ At decision ☐ After	RACI-014 Section I line initialled by the PI
		☐ Before ☐ At decision ☐ After	
		☐ Before ☐ At decision ☐ After	
		☐ Before ☐ At decision ☐ After	
		☐ Before ☐ At decision ☐ After	
		☐ Before ☐ At decision ☐ After	

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GLENOS-01 / 014	POT-014 / v0.2	CCDM-014 / v0.2 CCDM-014 / v0.2	1 / 1

B2 CCDM tasks retained by the PI or under active PI review

Copy from the CCDM every task coded C1 (supervision S0) or reviewed at level S1, and mark when the PI acts on each.

CCDM task ID	Task or decision (as in the CCDM)	Code / supervision	When the PI acts	Evidence of PI involvement (record)
T01	Final eligibility decision	C1 / S0	☑ Before ☐ At decision ☐ After	PSEAR decision signed by the PI before enrollment
T02	Final clinical safety classification and causality	C1 / S0	☐ Before ☑ At decision ☐ After	SART causality section signed by the PI
T03	Reverse shoulder arthroplasty implantation per surgical technique	C2 / S1 + S2	☐ Before ☑ At decision ☐ After	ICDL deviation entry countersigned; SLCT case review
T05	Postoperative radiographs per imaging charter (true AP, axillary)	C2 / S1 + S2	☐ Before ☐ At decision ☑ After	IQL entries read at the scheduled review
T09	Device accountability, returns and explant handling	C2 / S1 + S2	☐ Before ☑ At decision ☐ After	Explant record countersigned; sponsor device route
			☐ Before ☐ At decision ☐ After	
			☐ Before ☐ At decision ☐ After	
			☐ Before ☐ At decision ☐ After	
			☐ Before ☐ At decision ☐ After	
			☐ Before ☐ At decision ☐ After	
			☐ Before ☐ At decision ☐ After	
			☐ Before ☐ At decision ☐ After	
			☐ Before ☐ At decision ☐ After	
			☐ Before ☐ At decision ☐ After	
			☐ Before ☐ At decision ☐ After	

If more rows are needed, reprint this sheet, number it and file all sheets with the approved plan.

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GLENOS-01 / 014	POT-014 / v0.2	CCDM-014 / v0.2 CCDM-014 / v0.2	1 / 1

C Event-triggered escalation pathway

These events reach the PI at once, whatever the review schedule. Add the study's own triggers in the empty line and keep the same list in the escalation column of the CCDM.

No.	Trigger event	Who notifies (Role ID)	PI notified within	Immediate action	Record
1	**Surgery by a surgeon who has not been released (SCR or TRE)**	P-03	Before incision	Do not start the case; call the PI	ICDL pre-case check
2	**Missing implant lot number or UDI record**	P-03	Before wound closure	Hold the case record; photograph the labels	ICDL
3	**Unplanned component substitution**	P-02	Before wound closure	Record the reason; the PI decides on continuation	ICDL; PD record
4	**Substantial uncertainty about eligibility**	P-06	Before enrollment	Pause enrollment and refer to the PI	PSEAR
5	**Delayed SAE reporting**	P-06	Same day	PI reviews and reports to the sponsor	SART
6	**Missed critical imaging window**	P-04	1 working day	Repeat while possible; inform the sponsor	IQL; PD record
7	**Major PD affecting endpoint integrity**	P-07	1 working day	PI classifies and reports the PD	PD record; CAPA if recurrent
8	**Revision, explant, reoperation or readmission**	P-06	24 hours	PI assesses; explant route if applicable	SART; SCML
9	**Serious adverse event, serious adverse device effect, or a device deficiency that could have led to one**	P-06	Same day	PI assesses seriousness and causality; sponsor notified within the CIP interval	SART
10	Intraoperative fracture (CCDM T03 trigger)	P-02	Before wound closure	Protect the participant; document the fixation	ICDL; SART

If more triggers are needed, reprint this sheet, number it and continue the numbering.

D Review cycle and meeting architecture

For each setting, name who attends by Role ID, how often it meets, which records it reads and where its record is kept. The scheduled review is the one recorded in Part 2.

Setting	Participants (Role IDs)	Cadence / when	Sources read	Record
Weekly operational huddle	P-06, P-03, P-04, P-05	Mondays, 30 minutes	Upcoming cases and visit windows; open ICDL and IQL issues	Huddle notes in the investigator site file
Scheduled oversight review (the POT review cycle)	P-01, P-06, P-07, P-04	First Tuesday of each month	SART, SCML and CSST, SLCT, PIR, FUCR, IMC, CAPA and SRR; escalations received	POT Part 2
Case-triggered review	P-01, P-02, P-06 and others as needed	Within 2 working days of a Section C event	Case records (ICDL, SART, IQL)	POT Part 2 (triggered)

The PI sets the interval of the scheduled review: shorter when many participants are active, longer when few are. Huddles need no POT record. Hold an earlier review after a serious adverse device effect, a cluster of complications, a protocol amendment or the departure of a key person.

If the PI also operates on study participants: decisions on the PI's own authorization and performance are taken by (Role ID, or a party outside the site), agreed before the first case: Not applicable: the PI does not operate in GLENOS-01. If this changes, decisions on the PI's own authorization go to the sponsor's medical monitor.

E Plan approval and revision

Reason for this revision

- ☐ Initial plan
- ☑ CCDM revision
- ☐ RACI change (functions or holders)
- ☐ New trigger or event
- ☐ MRL milestone finding
- ☑ CAPA
- ☐ Sponsor or CIP change
- ☐ Other

Supersedes (plan ID / revision) and summary of change: Supersedes POT-014 v0.1 (15 Sep 2026). Trigger 3 (unplanned component substitution) now reaches the PI before wound closure, after PD-014-07 and CAPA-014-2026-003; Sections B1 and B2 aligned with the CCDM v0.2 task IDs; trigger 10 added.

Prepared by

Name / role: Maria Silva / Study Coordinator

Signature / date:

M. Silva 29 Sep 2026

PI / accountable approver

Name / role: Dr Alex Morgan / PI

Signature / date:

A. Morgan 30 Sep 2026

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Part 2 - Oversight review record

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POT plan ID / revision	Review ID	Review date	Sheet number / total sheets
POT-014 / v0.2	POT-014-R-2026-10	6 Oct 2026	1 / 1

I Review identification

Review type	Period covered	Attendees (Role IDs)	Previous review ID
☑ Scheduled review ☐ Triggered review (Section C no. ___) ☐ Case-triggered review	1 to 30 Sep 2026; escalations to 6 Oct	P-01, P-06, P-07, P-04	POT-014-R-2026-09 (start-up, 15 Sep)

II Cycle sources and PI decision

Read every record listed and mark one decision per line. **Monitor**: check again at the next review. **Action routed**: write the ID of the record that received the action. **Escalated**: the matter needs a decision outside the site, or a Hold in Section III.

Source	Version or period read	Signal since last review	PI interpretation	Decision	Destination record and ID
1 · SART Open safety cases and reporting status	SART log, 6 Oct 2026	☑ Yes ☐ No	SART-014-002: greater tuberosity fracture in case 2, fixed. PI: procedure-related, not device-related; non-serious.	☐ No signal ☐ Monitor ☑ Action routed ☐ Escalated	SCR-014-03 log (6 Oct); SLCT-014 case 2
2 · SCML and CSST New complications; signals by category, component and surgeon	SCML-014, 1 entry; CSST not yet due	☑ Yes ☐ No	One complication indexed (SART-014-002); no pattern with two cases.	☐ No signal ☑ Monitor ☐ Action routed ☐ Escalated	CSST at five cases
3 · SLCT Each surgeon's recent cases, read after the complications	SLCT-014, Dr Teixeira, cases 1 and 2	☑ Yes ☐ No	Case 2 read with its fracture; operative time within the comparator for its complexity. Too early for a pattern.	☐ No signal ☑ Monitor ☐ Action routed ☐ Escalated	SLCT-014 review at case 5
4 · PIR Indicator status against its rules	PIR, September 2026	☐ Yes ☑ No	Two surgeries in the period; counts too small to read a trend.	☑ No signal ☐ Monitor ☐ Action routed ☐ Escalated	N/A
5 · FUCR Visits in window and missing components	FUCR-014-2026-09	☐ Yes ☑ No	No follow-up visit due yet.	☑ No signal ☐ Monitor ☐ Action routed ☐ Escalated	N/A
6 · IMC Findings of internal monitoring	IMC-014-2026-09	☑ Yes ☐ No	Liner size missing in the case 1 ICDL, corrected the same day from the label.	☐ No signal ☑ Monitor ☐ Action routed ☐ Escalated	Recheck in IMC-014-2026-10
7 · CAPA Open actions and effectiveness reviews due	CAPA log, 6 Oct 2026	☑ Yes ☐ No	CAPA-014-2026-003 implemented: substitution reached the PI before closure in case 2. Effectiveness review after three more cases.	☐ No signal ☑ Monitor ☐ Action routed ☐ Escalated	CAPA-014-2026-003
8 · SRR New risks and triggers met since the last review	SRR-014 v0.3, 29 Sep 2026	☑ Yes ☐ No	New risk SRR-014-12: low recent reverse volume of the P-02 backup; controlled by SCR-014-03.	☐ No signal ☑ Monitor ☐ Action routed ☐ Escalated	SRR-014-12
9 · Escalations received From the CCDM, SCR, RACI or another route	Trigger 10 (2 Oct); RACI-014 interim cover (5 Oct)	☑ Yes ☐ No	The fracture reached the PI before closure, as planned. P-04 covered by P. Lopes from 6 to 17 Oct, with no backup during the cover.	☐ No signal ☐ Monitor ☑ Action routed ☐ Escalated	RACI-014 Section I (5 Oct); SRR-014-13
10 · Sponsor correspondence	Sponsor monitoring visit letter MV-01	☐ Yes ☑ No	No site-level finding.	☑ No signal ☐ Monitor ☐ Action routed ☐ Escalated	N/A

III PI conclusion and sign-off

☐ No action needed	☑ Actions routed	☐ Escalated to the sponsor	☐ Hold
No source showed a signal that needs a route.	Actions sent to their owning records (Section II).	A matter needs the sponsor's decision or reporting route.	Inclusion or surgery paused until the matter is resolved.

Mark the most restrictive decision that applies across Section II. Write in "Rationale" why, and carry anything not settled today to the next review.

Rationale

Two surgeries in the period. The only safety event reached the PI as planned and is related to the procedure; the conditional scope of Dr Teixeira stays as recorded in SCR-014-03. Imaging depends on one radiographer until 17 Oct, and that risk was added to the SRR.

Items carried to the next MRL milestone

MRL-014-M2 (first enrolled cases): confirm ICDL completeness after the liner-size correction, and read the SLCT at Dr Teixeira's fifth case.

Open items for the next review

SRR-014-13 until the P-04 holder returns (17 Oct). IMC recheck of the ICDL fields. CSST and CAPA-014-2026-003 effectiveness review once five cases are done.

Next review date	Linked MRL milestone	Linked records opened at this review
3 Nov 2026	MRL-014-M2	SRR-014-12, SRR-014-13

PI / accountable approver

Name / role: Dr Alex Morgan / PI

Signature / date:

A. Morgan 6 Oct 2026

Supporting reviewer (if applicable)

Name / role: Maria Silva / Study Coordinator

Signature / date:

M. Silva 6 Oct 2026

Reprint Part 2 (pages 5 and 6) for each review, number the sheets and file them with the approved plan.